

08. Vaginitis Treatment, Recurrence, and Partner Management

Companion reading handout for simultaneous listening.

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The prescription is the easy part.

The hard part is knowing what failure means.

When a patient says, "It came back," that sentence can mean several different things. It may mean bacterial vaginosis recurred because the vaginal ecosystem did not return to durable lactobacillus dominance. It may mean trichomoniasis was treated in the patient but not in the partner, so reinfection occurred. It may mean *Candida albicans* was treated correctly but the patient is having recurrent disease. It may mean the species is not *Candida albicans*. It may mean the microscopy was wrong, the culture represented colonization, the patient self-treated before testing, or the real diagnosis was contact dermatitis, vulvar dermatosis, genitourinary syndrome of menopause, urinary infection, cervicitis, or pelvic inflammatory disease.

So do not treat recurrence as a command to repeat the last prescription.

Treat recurrence as a diagnostic question.

The dominant model for this lecture is this: treat the biologic problem, and when treatment fails, investigate the pattern of failure.

BV is an ecosystem problem.

Trichomoniasis is a transmissible protozoal infection.

Candidiasis is a yeast-inflammation syndrome shaped by host factors, severity, recurrence, and species.

The treatment logic follows from those three biologic identities.

One scope note before we start: this lecture follows the ACOG vaginitis guideline for nonpregnant patients. Pregnancy, significant medication allergy, and unusual comorbidity can change the risk-benefit calculation and should be handled with pregnancy-specific or specialist guidance.

Bacterial vaginosis

In Lecture 7, we framed bacterial vaginosis as dysbiosis, not a classic invasive inflammatory infection. Lactobacilli decrease. Facultative and anaerobic organisms increase. pH rises. Clue cells appear. Odor and thin homogeneous discharge may occur, but many patients are asymptomatic.

Treatment is recommended for symptomatic BV.

The mechanism is not sterilizing the vagina. The mechanism is reducing overgrowth of endogenous facultative and anaerobic bacteria so the ecosystem can move back toward lactobacillus predominance.

That sentence matters. BV treatment is not like removing one named bacterium from an otherwise normal environment. The resident should picture a disturbed ecology being pushed back toward balance.

ACOG recommended regimens include three first-line options.

Metronidazole five hundred milligrams orally twice daily for seven days.

Metronidazole gel zero point seven five percent, five grams intravaginally once daily for five days.

Or clindamycin cream two percent, five grams intravaginally at bedtime for seven days.

Alternative regimens include secnidazole two grams orally once, tinidazole two grams orally once daily for two days, tinidazole one gram orally once daily for five days, clindamycin three hundred milligrams orally twice daily for seven days, or clindamycin ovules one hundred milligrams intravaginally at bedtime for three days.

That is the list. Now make it clinically usable.

Because the recommended therapies have broadly comparable safety and efficacy profiles, selection should be individualized. Ask about patient preference, cost, adherence, convenience, ease of use, previous response, and previous adverse effects.

For example, if oral metronidazole caused significant gastrointestinal adverse effects before, intravaginal metronidazole gel may be more tolerable. If adherence to a week of medication is unlikely, convenience may matter. If the patient is using intravaginal medication, she may want to avoid tampons during treatment so medication dispersion is not impaired.

For oral nitroimidazoles, drug manufacturers recommend avoiding alcohol during treatment and for twenty four hours after metronidazole or seventy two hours after tinidazole, because of the theoretical concern for a disulfiram-like reaction.

Clindamycin ovules have another counseling point: the oleaginous base may weaken latex or rubber products, including condoms and vaginal contraceptive diaphragms. Use of those products within seventy two hours after clindamycin ovule treatment is not recommended.

Patients should also avoid sexual activity during BV treatment unless condoms are used. But here is the important distinction: this is not the same as saying partners need routine treatment.

For BV, routine sex partner treatment is not supported. Data do not show that partner treatment improves relapse or remission. That can feel counterintuitive because BV is associated with sexual activity and is uncommon in people who have never been sexually active. But association is not the same as a single transmitted pathogen.

Because BV is associated with increased susceptibility to sexually transmitted infections, the clinical visit is also a moment to think about appropriate STI testing, including HIV testing, based on guideline context and the patient's risk.

The resident reflex is this: BV treatment restores an ecosystem; it is not partner treatment by default.

Recurrent BV

If symptoms resolve after treatment, routine rescreening for BV is not necessary.

But recurrence is common. ACOG reports recurrence in up to thirty percent of patients within three months and fifty eight percent within twelve months.

Those numbers are worth teaching because they change counseling. If you tell the patient only, "Take this and you should be fine," you have not prepared her for the real natural history. A better counseling line is: "This usually responds, but recurrence is common. If symptoms return, come back so we can confirm the diagnosis and choose the next strategy."

Potential contributors to recurrent BV include douching, frequent sexual activity, previous BV, persistence of pathogenic bacteria, and failure to reestablish lactobacillus-predominant flora.

Recurrent BV is generally defined as at least three documented, separate episodes in one year.

For these patients, after treating the acute episode, twice-weekly suppressive metronidazole gel for sixteen weeks may be offered. Changing the antibiotic or extending the antibiotic course may also help.

But do not jump straight to suppression without confirming the pattern. If the patient says BV keeps returning, ask: were the episodes documented? Was pH elevated? Were clue cells present? Was there whiff? Were symptoms compatible? Was there trichomoniasis testing? Is there candidiasis, irritant exposure, or a vulvar disease explaining symptoms?

The recurrence question is not only, "Which longer prescription?" It is, "Are we still treating the right disease?"

Trichomoniasis

Trichomoniasis is caused by *Trichomonas vaginalis*. It is a sexually transmitted protozoal infection. That means treatment has two targets: the organism in the patient, and the reinfection loop between partners.

For nonpregnant women with uncomplicated trichomoniasis, ACOG lists metronidazole five hundred milligrams orally twice daily for seven days as the recommended regimen. Tinidazole two grams orally once is an alternative.

This is an important update from the older mental model in which single-dose metronidazole was often treated as the default. ACOG notes randomized-trial data showing that the seven-day metronidazole course is more effective for women.

Oral nitroimidazoles are the recommended treatment. Metronidazole gel is not effective for *Trichomonas vaginalis* infection.

That line should be unforgettable.

Metronidazole gel is not trichomonas treatment.

The reason is simple. Trichomoniasis requires systemic nitroimidazole therapy. A local vaginal gel does not reliably eradicate the protozoal infection.

Tinidazole may have fewer gastrointestinal adverse effects for some patients but may be more expensive. The same manufacturer alcohol-avoidance counseling applies: avoid alcohol during nitroimidazole therapy, and for twenty four hours after metronidazole or seventy two hours after tinidazole.

If a patient has a true metronidazole allergy, do not improvise with ineffective local therapy. Referral for desensitization is recommended.

Partner management

Whenever trichomoniasis is confirmed, current sex partners should receive presumptive therapy. The patient and partners should avoid sexual activity until they have completed therapy and are asymptomatic. A practical counseling frame is: avoid sex until treatment is completed, symptoms have resolved, and it has been a full seven days since the last antibiotic dose.

Retest the patient within three months because recurrence and reinfection are common.

Here is where residents must be careful with language.

Persistent trichomoniasis symptoms after treatment do not automatically mean drug resistance.

First ask about adherence. Did she take the full regimen? Did vomiting occur? Did she stop because of adverse effects? Did the partner receive treatment? Was there sex before treatment was complete? Was there a new partner?

Only after adherence and reinfection have been carefully assessed should resistance move up the list.

ACOG notes that high-level metronidazole resistance is rare, although low-level in vitro resistance may be as high as four to ten percent. In suspected resistance, tinidazole may be effective. If retreatment fails and adherence is assured, a culture of the potential resistant isolate can be sent to a reference laboratory for susceptibility testing.

The resident reflex is this: trichomonas treatment is patient plus partner plus retesting.

Candidiasis

Candidiasis is where many treatment errors happen because the drug is familiar and easy to prescribe. But familiar treatment can hide sloppy classification.

Before treating, classify candidiasis as uncomplicated or complicated.

Uncomplicated vulvovaginal candidiasis requires all of the following: sporadic or infrequent episodes, mild to moderate symptoms, suspected or proven *Candida albicans*, and a non-immunocompromised host.

For uncomplicated disease, intravaginal azole therapy or oral fluconazole is recommended. The commonly used oral regimen is fluconazole one hundred fifty milligrams orally once.

Many intravaginal azoles are available, including over-the-counter options. The exact product can be selected based on patient preference, cost, convenience, adherence, ease of use, prior response, and prior adverse effects. In uncomplicated disease, symptomatic relief and mycologic cure are greater than ninety percent.

Topical therapy can cause local burning or irritation. Oral fluconazole is convenient and generally well tolerated, but can cause systemic adverse effects such as gastrointestinal intolerance, headache, and liver-function test elevations, usually mild and self-limited.

Complicated candidiasis

Complicated disease includes any of the following: recurrent episodes, severe symptoms or findings, suspected or proven non-*albicans* *Candida*, diabetes, HIV, other immunocompromising conditions, debility, or immunosuppressive therapy such as corticosteroids.

This classification is not academic. It tells you when a single tablet is too simple.

Recurrent vulvovaginal candidiasis means four or more infections in twelve months.

The diagnosis should be objective. Do not accept "I had four yeast infections" unless yeast was actually demonstrated or appropriately documented. Recurrent disease should be supported with culture and speciation, because the plan depends on the organism.

For recurrent *Candida albicans*, the usual logic is induction followed by suppression. First control the acute infection. Then suppress recurrence.

A common suppression strategy is fluconazole one hundred fifty milligrams weekly for six months. ACOG reports that prolonged fluconazole therapy controlled more than ninety percent of recurrent symptomatic episodes during therapy, with prolonged protective effect in about fifty percent of patients with recurrent *Candida albicans*.

That is a useful counseling point. Suppression works very well while the patient is taking it, but it is not a guaranteed permanent cure. The aim is control and reduction of recurrence, not a promise that the disease can never return.

For patients who cannot or do not want to take fluconazole, prolonged intermittent topical azole therapy can be used, such as clotrimazole five hundred milligrams weekly or two hundred milligrams twice a week.

Severe candidiasis

Severe candidiasis is not the same category as recurrent candidiasis. Severe disease is about intensity of vulvar findings: erythema, erosion, fissure, and edema.

These patients need a longer acute course. Options include ten to fourteen days of topical intravaginal azole therapy, or oral fluconazole every three days for two or three doses, such as days one, four, and seven.

Suppression is not automatically needed just because the episode is severe. If the patient has severe disease but not recurrent disease, the immediate issue is adequate acute treatment.

That distinction helps residents avoid a common mistake: confusing severity with recurrence.

Severe candida needs more acute treatment. Recurrent candida needs documented diagnosis and a suppression strategy.

Non-albicans Candida

Non-albicans *Candida* causes a minority of vulvovaginal candidiasis, roughly five to ten percent, but it matters because it often behaves differently. *Candida glabrata* is

the classic example.

Non-albicans species are less likely to respond to topical imidazoles or oral fluconazole. They should be suspected when symptoms continue after treatment for presumed uncomplicated candidiasis.

The first move is culture and speciation. Do not keep giving fluconazole blindly to an organism that may be intrinsically less susceptible.

For *Candida glabrata* and other atypical *Candida* species, intravaginal boric acid six hundred milligrams daily for at least fourteen days can be effective.

But boric acid requires serious counseling.

It can be fatal if swallowed orally. The patient must understand: intravaginal use only, keep it away from children, and use reliable contraception. This is not a casual wellness product. It is a vaginal medication with oral-poisoning risk.

Topical flucytosine five grams nightly for two weeks is another option for *Candida glabrata*, but cost and access can be limiting. Persistent non-albicans disease after boric acid should prompt referral to a subspecialist.

The resident reflex is this: candida treatment begins with classification.

Uncomplicated, recurrent, severe, or non-albicans.

Now step back and compare partner treatment across the three major causes.

Treat current sex partners for confirmed trichomoniasis.

Do not routinely treat partners for bacterial vaginosis.

Do not routinely treat partners for uncomplicated vulvovaginal candidiasis.

This is not a memorized list. It follows from mechanism.

Trichomoniasis is a sexually transmitted infection where untreated partners can maintain transmission and reinfection. BV is an ecosystem disorder associated with sexual activity but not caused by one partner transmitting one pathogen in a simple way. Uncomplicated candidiasis is usually not managed as a partner-transmission problem.

Asymptomatic findings

Do not treat asymptomatic *Candida* found on Pap.

Do not treat asymptomatic BV found on Pap.

Pap tests are not reliable tests for vaginitis. They are cervical cytology screening tests. If a symptomatic patient has a Pap report suggesting *Candida* or BV, confirm with appropriate diagnostic testing. If an asymptomatic patient has incidental *Candida* or BV on Pap, treatment is not indicated.

Trichomonas on Pap should be handled differently. Because of the clinical and partner implications, a Pap report suggesting trichomonas should prompt confirmatory testing, and confirmed trichomoniasis should be treated.

Self-treatment and over-the-counter antifungals

Over-the-counter azoles can be effective when the diagnosis truly is uncomplicated candidiasis. The problem is not that the medication never works. The problem is that self-diagnosis is often inaccurate, and symptoms are nonspecific.

If the patient has used a nonprescription antifungal and still has symptoms, she needs clinical evaluation. Topical antifungals can cause local burning and irritation. They can also delay the correct diagnosis if the actual disease is bacterial vaginosis, trichomoniasis, urinary infection, cervicitis, pelvic inflammatory disease, contact dermatitis, or vulvar dermatosis.

The resident should ask a precise question: "Was yeast ever objectively shown?"

If the answer is no, then the problem may not be recurrent yeast. It may be recurrent treatment of the wrong disease.

Probiotics and nonmedical therapies

ACOG does not recommend vaginal or oral probiotics or nonmedical therapies for treatment or prevention of vaginitis. That includes the idea of using lactobacillus products to treat or prevent bacterial vaginosis or vulvovaginal candidiasis. Other nonmedical approaches proposed for candidiasis, such as yogurt, garlic, tea tree oil, low carbohydrate diets, and douching, do not have sufficient supporting evidence, and some can cause harm or delay proper care.

This is one place where clinicians may encounter older local guidance or patient-facing advice that sounds more permissive about probiotics. For this course, the current ACOG source is the anchor: do not present probiotics as proven treatment or prevention.

Now bring the lecture together through a few management cases.

Case one.

A patient has symptomatic BV. She previously stopped oral metronidazole because of nausea. Her diagnosis is confirmed. She is not asking for partner treatment; she is asking for something tolerable.

The biologic target is still BV dysbiosis. A reasonable move is intravaginal metronidazole gel or clindamycin cream, depending on preference, cost, and prior

response. The key is not to call her "nonadherent" before choosing a regimen she can realistically complete.

Case two.

A patient has documented BV four times in one year. She also douches because she thinks it prevents odor.

This is recurrent BV. Treat the acute episode, counsel strongly against douching, and consider suppressive metronidazole gel twice weekly for sixteen weeks. The teaching point is that recurrence prevention is not only a longer prescription. It also means removing behaviors that repeatedly disturb the ecosystem.

Case three.

A patient with confirmed trichomoniasis completed only three days of metronidazole because she felt better. Her partner was never treated. Symptoms are back.

Do not call this resistance. This is failure of the treatment system: incomplete therapy plus untreated partner exposure. Treat appropriately, treat the partner, counsel no sex until therapy is completed and symptoms resolve, and retest within three months.

Case four.

A patient has four proven *Candida albicans* episodes in twelve months.

This is recurrent vulvovaginal candidiasis. Confirm the organism and then use induction followed by suppression, commonly fluconazole one hundred fifty milligrams weekly for six months after acute control. Counsel that suppression controls episodes very well during therapy, but recurrence after stopping can occur.

Case five.

A patient has persistent symptoms after fluconazole. Culture shows *Candida glabrata*.

This is not ordinary uncomplicated *Candida albicans*. Stop repeating the same fluconazole reflex. Consider intravaginal boric acid six hundred milligrams daily for at least fourteen days with explicit oral-ingestion safety counseling and reliable contraception counseling, or specialist referral when disease persists or therapy is not appropriate.

The final synthesis

BV treatment restores an ecosystem. Treat symptomatic disease, choose a regimen the patient can complete, do not routinely treat partners, and recognize that recurrence is common because restoring durable lactobacillus dominance is not always easy.

Trichomoniasis treatment eradicates a sexually transmitted protozoal infection. Use systemic nitroimidazole therapy, not gel. Treat partners. Avoid sex until treatment is

complete and symptoms resolve. Retest within three months. Before assuming resistance, check adherence and reinfection.

Candidiasis treatment depends on classification. Uncomplicated disease can be treated with intravaginal azoles or one oral fluconazole dose. Recurrent disease needs objective diagnosis and suppression. Severe disease needs longer acute therapy. Non-albicans disease needs culture, speciation, and different treatment logic.

And the final resident reflex is this.

When vaginitis keeps coming back, do not simply repeat the same prescription. Ask what failed: diagnosis, adherence, partner management, organism, host, species, behavior, or ecosystem.